

CVS Health

Medication Tracker

A unified daily dose log and adherence hub in the CVS Health app

Author	Courtney
Status	Draft v1.0 (Portfolio)
Date	June 2026
Document type	Product Requirements Document
Product area	Digital Pharmacy and Adherence

1. Executive Summary

Medication nonadherence affects an estimated 30 to 50 percent of patients and is a leading, preventable driver of poor health outcomes and avoidable cost. The CVS Health app already offers refill reminders and a reminder feature (MedRemind), but these tools are fragmented and reminder first. They do not give patients or caregivers a single place to log doses, confirm whether today's medications were actually taken, and track adherence over time.

The Medication Tracker introduces a unified daily medication schedule, one tap taken or skipped logging, an adherence streak and history, and an optional caregiver view. It turns scattered reminders into a coherent adherence system that connects directly to CVS refill timing and pharmacist support.

Why now

Adherence is strategically central to CVS Health across pharmacy, Caremark, and Aetna. Higher adherence improves Star Ratings and clinical outcomes, increases refill volume and on time refills, and deepens daily engagement with the app as the household health hub. A patient who logs doses every day is a retained, high value pharmacy customer.

2. Problem Statement

User problem

Patients managing multiple medications struggle to remember whether they took a dose, especially with complex regimens. App reviews describe users jumping from two prescriptions to ten or more after a hospital stay, with no simple way to answer the everyday question of whether the morning pills were taken. Reminders fire, but there is no log, no confirmation, and no trend. Caregivers managing a parent's or child's medications have even less visibility.

Business problem

Nonadherence drives missed refills, poorer outcomes, and lower Star Ratings, and an estimated one third of prescriptions are never filled at all. Reminder only tooling does not close the loop between

intending to take a medication and actually taking it, leaving measurable adherence and refill revenue unrealized.

3. User Personas

Persona	Snapshot	Primary pain point
Polypharmacy Patient (Robert, 67)	Manages ten or more medications after a recent hospitalization.	Loses track of doses and fears accidentally doubling up.
Busy Caregiver (Denise, 48)	Manages her father's medications remotely while working full time.	No way to confirm he took his evening dose.
Chronic Condition Manager (Priya, 34)	Lives with diabetes and wants a reliable daily routine.	Reminders alone do not create real accountability.
New Prescription Patient (Marcus, 29)	First time on a daily medication after a new diagnosis.	Forgets easily and is unsure of the schedule.

4. Goals and Success Metrics

Qualitative goals

- Close the loop from reminder to confirmed dose.
- Give caregivers peace of mind and a reliable remote view.
- Connect daily adherence to timely refills and pharmacist support.

Quantitative targets

Metric	Baseline	Target	Measurement
Daily dose logging rate (enrolled)	0%	50%	Feature telemetry
Measured adherence vs non users	Baseline	+10 points	Pharmacy claims (PDC)
On time refill rate (tracker users)	TBD	+12%	Refill data
Caregiver linkage adoption	0%	20%	Feature telemetry
30 day feature retention	TBD	40%	Engagement logs

- **North Star metric:** confirmed doses logged per enrolled patient per week.

5. Scope

In scope

- Import active prescriptions from the patient's CVS pharmacy profile into a daily schedule.
- One tap dose logging: taken, skipped, or snoozed.
- Today view with clear taken versus pending status.
- Configurable per medication reminders, plus adherence streak and history.
- Manual add for over the counter medications and supplements.
- Optional, consent based caregiver view, and a low supply refill prompt.

Out of scope

- Clinical dosing advice or interaction diagnosis beyond the existing interaction checker.
- Automatic dispensing or schedule changes without patient confirmation.
- Integration with non CVS pharmacies at launch.
- Insurance or billing changes.
- Specialized controlled substance workflows beyond standard display.

6. Functional Requirements

ID	Requirement	Priority
FR1	Patient can import active CVS prescriptions into a daily medication schedule.	Must Have
FR2	Patient can log each scheduled dose as taken, skipped, or snoozed with one tap.	Must Have
FR3	App displays today's medications with clear taken or pending status.	Must Have
FR4	Configurable per medication reminders with time and frequency.	Must Have
FR5	Adherence history view with streak and weekly or monthly summary.	Should Have
FR6	Patient can manually add over the counter medications and supplements.	Should Have
FR7	A consenting patient can share their schedule and dose log with a designated caregiver.	Should Have
FR8	Caregiver receives an alert when a patient configured critical dose is missed.	Should Have
FR9	Tracker connects to refill status and prompts a refill when supply is low.	Should Have
FR10	Glanceable dose confirmation on a paired wearable.	Nice to Have
FR11	Adherence summary exportable to share with a provider.	Nice to Have

7. Non Functional Requirements

- **Privacy and compliance:** HIPAA compliant handling of protected health information; caregiver access is strictly consent based and revocable, with audit logging.
- **Reliability:** reminders must fire dependably; the dose log must persist offline and sync when reconnected.
- **Accessibility:** large tap targets and high contrast for older adults; full screen reader support; meets WCAG AA.
- **Performance:** the schedule loads in under two seconds and logging is instant, even offline.
- **Safety:** the feature never substitutes for medical advice; clear disclaimers; no schedule changes from claims data without patient confirmation.

8. Risks and Assumptions

Risks

- **Logging fatigue:** daily logging can feel like a chore. Mitigate with one tap entry and smart defaults.
- **False confidence:** an inaccurate log could imply adherence that did not happen. Mitigate with clear prompts and easy correction.

- **Consent complexity:** the caregiver feature raises PHI and consent requirements that must be handled rigorously.
- **Reminder reliability:** dependable notifications across operating systems is a known hard problem requiring careful engineering.

Assumptions

- Patients want confirmation and history, not just reminders.
- Caregivers will drive adoption for elderly and dependent patients.
- CVS pharmacy data can populate accurate daily schedules for most regimens.
- Better logging correlates with better real world adherence.

9. Proposed Timeline

Phase	Focus	Duration
Discovery	Patient and caregiver research; clinical and compliance review.	4 weeks
Design	Older adult and caregiver centered UX for schedule and logging.	5 weeks
Build (alpha)	Schedule, one tap logging, and reminders.	8 weeks
Beta	Caregiver linkage and refill tie in.	5 weeks
GA launch	Phased rollout with monitoring.	3 weeks

10. Open Questions

- How do we build an accurate dose schedule when instructions are ambiguous, such as as needed dosing?
- What defines a critical dose for caregiver alerts, and who sets it?
- Should adherence data feed Star Ratings programs, and what consent does that require?
- How do we avoid alarm fatigue while still flagging genuinely missed critical doses?
- Do we support household profiles, where one caregiver manages multiple patients?

11. Appendix

- **Market context:** nonadherence affects 30 to 50 percent of patients; roughly one third of prescriptions are never filled; CVS already offers MedRemind and refill reminders.
- **Related features:** drug interaction checker, ExtraCare, and Caremark and Aetna integration.
- **Compliance references:** HIPAA and applicable state pharmacy regulations.
- **Glossary:** PDC (proportion of days covered), adherence, polypharmacy, sig.